

MANAGEMENT OF OTHER COMMON SKIN CONDITIONS IN PRIMARY CARE

Key Family Physician: Dr Choong Siew Li

Specialist Advisor: Adj A/Prof Nisha Suyien Chandran (Senior Consultant, National University Hospital)

Acknowledgement for their inputs: NUHSP: Ms Waseemah Begum

Updated in January 2026 by Dr Choong Siew Li. Next review date: January 2029.

CONTENTS

Urticaria.....	3
Introduction	3
Management of Urticaria	3
Alopecia.....	4
Introduction	4
Causes of Alopecia.....	4
Common Acquired Pigmentary Disorders.....	6
Freckles	6
Post-inflammatory Hyperpigmentation (PIH).....	6
Vitiligo	6
Drug Rashes and Allergy Testing	7
Skin Growths and Tumours	8
Skin Tags, Seborrheic Keratoses, Sebaceous Hyperplasia, Syringomas	8
Suspected Basal Cell Carcinoma, Squamous Cell Carcinoma, Melanoma, Bowen’s Disease	8
Scars and Keloids	9
Keloids.....	9
Scars	9
References	9

URTICARIA

Introduction

Urticaria is a skin reaction pattern characterised by transient pruritic oedematous, erythematous lesions. Individual lesion typically lasts less than 24 hours with no post inflammatory hyperpigmentation.

Management of Urticaria

1. Patient Education

- Eliminate cause of urticaria (drugs / diet / physical environment)

2. Investigations

- Consider Full Blood Count, Erythrocyte Sedimentation Rate & Thyroid Function Test in chronic urticaria (> 6 weeks' duration)

3. H1- Anti-Histamines

- First line: 2nd generation H1- antihistamine: Cetirizine or Loratadine 10mg BD. Can be increased up to 4 times the standard dose.
- Second line: Switch to Fexofenadine 180mg BD or Bilastine 20mg OD/BD if not better (*both drugs are [not available in NUP](#)). Can be increased up to 4 times the standard dose.
- Consider adding another second-generation H1-antihistamine.
Consider adding 1st-generation H1-antihistamine at bedtime (Chlorpheniramine 4mg ON, Hydroxyzine 10-25mg ON).
- Suggested treatment duration: At least 2 weeks, or 2 months (if chronic)
- Consult pharmacist for the necessary dose adjustments for patients with renal impairment.

4. H2 Anti-Histamines

- If symptoms not controlled by H1 anti-histamines, clinicians can consider adding
 - Famotidine 20 mg BD (Renal dose adjustment required), OR
 - Cimetidine 400 mg BD ([*Not Available in NUP](#))
- There is however weak evidence supporting the combination of H1 and H2 antihistamines.

5. Oral Corticosteroids

- Short course of prednisolone (0.5-1mg/kg/day [max 60mg/day]) for 3 - 7 days if symptoms are severe in acute urticaria.

6. Specialist Consultation

- Dermatologist for refractory urticaria, or if urticarial vasculitis suspected (pain, purpura / pigmentation, lesions lasting > 24 hours, atypical features)
- A&E referral if severe angioedema with airway compromise or hemodynamic instability

ALOPECIA

Introduction

Alopecia is the partial or complete absence of hair from areas of the body where it normally grows. In non-scarring alopecia, the hair follicles are preserved and visible. In scarring alopecia, the hair follicles are obliterated and destroyed.

Causes of Alopecia

- 1) Disturbance of the hair follicle:
 - Androgenetic alopecia (male / female pattern hair loss)
 - Alopecia areata
- 2) Disturbance of the hair cycle:
 - Telogen effluvium
- 3) Physical factors:
 - Trauma (e.g., from deliberate pulling or prolonged pressure on scalp)
 - Traction (e.g., from hair styling)
- 4) Skin disease:
 - Fungal infection
 - Inflammatory skin disease that caused scarring (e.g. discoid lupus, lichen planus)
- 5) Systemic factors or diseases
 - Drugs
 - Endocrine disorders

6) Hair shaft abnormalities

- Genetic disease
- Damage by hair treatment

Management of Some Causes of Alopecia

Consider performing laboratory (e.g. FBC, Iron panel, Thyroid function test) for patients presented with new onset diffuse, non-scarring hair loss without a clear cause.

1. Androgenic Alopecia: “Patterned” alopecia with frontal recession (males), thinning of the crown and vertex, family history

- First line: OTC Minoxidil (2%, 3%, 5%) Solution BD
 - Female: 2% solution
 - Male: 5% solution recommended (If it causes irritation, can switch to lower strengths)
 - To continue indefinitely
- Hair wig
- Specialist consultation
 - Refer to dermatologist for finasteride (males) / spironolactone (females).
 - Patients can be initiated at specialist level and stepped down to primary care to continue treatment of Finasteride 1mg OD.
 - Please monitor for any adverse reactions of Finasteride during the follow-up, such as sexual dysfunction (e.g., impotence, decreased libido, ejaculatory disorder), gynecomastia, breast tenderness, prostate cancer.
 - Cosmetic procedures such as hair transplant

2. Alopecia Areata: Localised non-scarring patchy alopecia

- 1st line: Betamethasone 0.1% scalp lotion OM can be started in primary care. Concurrently, refer to dermatologist for management such as intralesional corticosteroid or topical immunotherapy.
- Minoxidil (2 - 5%) Solution BD (adjunct)
- Topical corticosteroids (less efficacious)

3. Telogen Effluvium

- Diffuse hair loss of relatively short duration
- Detailed history regarding underlying cause (recent delivery, severe stress, recent febrile illness such as dengue)
- Spontaneously improved within 6 – 9 months and treatment of the underlying cause

4. Scarring Alopecia

- Refer to dermatologist for further assessment and management.
- Trauma, infections, or inflammatory skin diseases can cause scarring alopecia.
 - Provide advice against tight curls if traction alopecia is suspected.
 - Treat existing scalp infections [i.e. bacterial (boils and abscesses), fungal or viral (herpes zoster)] which can lead to scarring alopecia.

COMMON ACQUIRED PIGMENTARY DISORDERS

Freckles

- **Encourage** sun protection.
- No urgency to refer as conditions are cosmetic and not subsidized.

Post-inflammatory Hyperpigmentation (PIH)

- Hyperpigmentation usually preceded by inflammation of skin (e.g. eczema, acne, injury *etc.*) especially in patients with darker skin phototypes (III – VI). Post inflammatory hypopigmentation may also occur.
- Reassurance that PIH will take time to fade (months to a few years).
- Treat underlying cause of PIH i.e., optimise treatment of eczema, acne.
- Encourage emollients and sun protection including regular use of sunscreen.

Vitiligo

- Patients with vitiligo presents with depigmented macules and/or patches which can be focal, segmental, or mixed. These will fluoresce under bedside Wood's lamp examination.
- Assess patient's severity and whether patient has stable or unstable vitiligo (stable disease: no increase in size of existing lesions and absence of new lesions are noted in the previous 3 to 6 months).

- Goals of treatment for vitiligo include stabilisation of disease, achieving repigmentation and prevention of recurrence. Expectations need to be managed regarding efficacy of all treatment methods.
- Vitiligo can be left alone if the patient does not want any treatment.
- For stable, localised disease:
 - First line: topical corticosteroids (mometasone furoate 0.1% cream OD or betamethasone 0.025% cream OD/BD) for up to 2 months. Watch for signs of cutaneous atrophy with topical steroids.
 - If no repigmentation occurs, consider referral to Dermatology for second line options (i.e. phototherapy).

DRUG RASHES AND ALLERGY TESTING

Diagnosis	Clinical Feature	Recommended Disposition
Suspected Food Allergy	• If a food-induced trigger is suspected or cannot be confidently excluded in a patient with a dermatosis.	1. Referral to allergist (at present, NSC does not offer food provocation testing). 2. Whilst awaiting referral, advise patient to keep a food diary.
Angioedema / anaphylaxis	• Angioedema / anaphylaxis ± urticaria • Drug aetiology usually within 24 hours of exposure of drug • Other non-drug causes to consider include: insect bite e.g. bee stings, infective trigger • Non-IgE mediate reactions can occur with a latency of hours to short days e.g., NSAID intolerance, opioids, contrast media, ACE inhibitors / ARBs – if in doubt consider stopping	1. Stop culprit drug if applicable. 2. Life /airway-threatening presentations should be sent to ED immediately, stop culprit drug if applicable. 3. To refer Dermatologist for evaluation if no obvious trigger. 4. If a food trigger is suspected, refer to allergist.

Diagnosis	Clinical Feature	Recommended Disposition
Exanthema	• Acute, generalised eruption • Can be maculopapular, macular, papular <i>etc.</i> • Usually, a viral trigger or a drug induced caused within a latency period of 4-14 days • Absence of warning signs of an evolving severe cutaneous adverse reaction ○ Mucositis, erosions / Nikolsky's sign, dusky target → EM-SJS-TEN (latency period of 4-30 days) ○ Fever (> 38.5°C), purpura, facial / earlobe oedema, scaling, induration → DRESS / DHS (latency period of 4-6 weeks but up to 3 months) ○ Pustules (especially of flexural sites) ± facial oedema → AGEP (latency period of 1-4 days)	1. Stop culprit drug if applicable. 2. If no warning signs, stop offending drug and treat symptomatically with moderate to high potency topical steroids, with review in a few days; no need to refer to Dermatologist at the outset. 3. If warning signs are present, refer to ED immediately. 4. If drug eruption suspected but unsure categorisation / management, refer to Dermatologist early.

SKIN GROWTHS AND TUMOURS

Skin Tags, Seborrheic Keratoses, Sebaceous Hyperplasia, Syringomas

- No urgency to refer as conditions are cosmetic / non-medical. To refer as private patient to Dermatologist.

Suspected Basal Cell Carcinoma, Squamous Cell Carcinoma, Melanoma, Bowen's Disease

- Refer to dermatologist for biopsy / excision and histology.

SCARS AND KELOIDS

Keloids

- First line: Intralesional steroid (*[Not Available in NUP](#)). Consider referral to Dermatologist.

Scars

- To refer to Dermatology clinic if patients request for scar treatment (non-subsidized).

REFERENCES

1. A comprehensive review on hair loss. Dermatological Society of Singapore. Accessed on 7 Feb 2022
2. Hair loss. Dermnet NZ. Updated December 2015. Accessed on 7 Feb 2022
3. Guidelines for Escalation of Therapy in the Primary Care Setting by National Skin Centre (Document shared by Dermatologist from National Skin Centre in April 2024)